When Parents Can Spare Time: Paid Sick Leave and Adolescent Mental Health*

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Abstract

Adolescents in the United States face rising rates of depression, anxiety, and suicidality, yet structural barriers such as parental work constraints continue to limit timely access to care. This paper provides new causal evidence that state Paid Sick Leave (PSL) mandates improve adolescent mental health. Using the staggered adoption of PSL mandates across states, we find that PSL laws increase the probability that an adolescent received any mental or behavioral health care in the past year by 2.3 percentage points (a 13.6% increase relative to the baseline mean of 17.1%) and raise parent-reported depression diagnoses by 1.5 percentage points (a 14.1% increase relative to the baseline mean of 10.9%). Complementary evidence from the Youth Risk Behavior Surveillance System shows that PSL mandates reduce self-reported depressive symptoms by 2.2 percentage points (7.2%) and passive suicidal ideation by 1.4 percentage points (8.6%), with no detectable effects on active suicidal ideation. Mechanism analyses suggest that these improvements operate primarily through enhanced parental time flexibility and increased engagement with the mental health care system. These findings highlight that labor protections enabling parental time flexibility can serve as an upstream intervention for adolescent mental health.

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1 Introduction

Children and adolescents in the United States face a growing mental health crisis, with rising rates of depression, anxiety, and suicidality over the past decade (Centers for Disease Control and Prevention, 2024; Xiang et al., 2024; Warnick and Kolade, 2024). Unmet mental health needs among youth remain widespread, particularly for conditions such as depression and behavioral disorders, which are often underdiagnosed and untreated until symptoms escalate (Lu, 2019; The Pew Charitable Trusts, 2024). These gaps in care have long-term consequences for educational attainment, employment, and health over the life course (Bubonya et al., 2017; Kondirolli and Sunder, 2022; Frijters et al., 2014; Bryan et al., 2022; Fletcher, 2013; Currie, 2024). One frequently overlooked structural barrier to timely detection and treatment is time constraints faced by parents, as mental health appointments and follow-ups typically occur during standard working hours. Limited time flexibility can delay early intervention, reduce treatment adherence, and exacerbate the severity of mental health conditions among children and adolescents, particularly during adolescence when symptoms often first emerge (Mitchell and Selmes, 2007; Hansen et al., 2021). In this study, we focus specifically on adolescents aged 12–17, for whom access to mental health care critically depends on parental involvement in scheduling, consent, transportation, and appointment attendance.

Policies that expand parents' ability to take time off for caregiving are thus particularly relevant in this context. Among such policies, paid sick leave (PSL) mandates have emerged as a key instrument to balance work and family responsibilities. PSL laws allow workers to take protected, paid time off for personal or family health needs without risking income loss or job security (Eisenberg et al., 2025; Seixas and Macinko, 2020; Callison et al., 2025). By increasing the availability of short-term, protected time away from work, PSL mandates may facilitate earlier health-seeking behavior, improve continuity of care, and strengthen family well-being. Prior studies suggest PSL mandates increase parental caregiving time and improve family stability (Assamidanov et al., 2025; Deza et al., 2025; Maclean and Pabilonia, 2025; Guo and Peng, 2025), yet their implications for adolescents' mental health remain largely unexplored. Whether these labor protections translate into measurable improvements in adolescents' mental health remains an open and policy-relevant question.

Unlike most high-income countries, the United States lacks a federal paid sick leave policy, leaving access to this benefit determined by state mandates or voluntary employer provision. As of 2024, 18 states and the District of Columbia have enacted statewide PSL mandates that typically provide 5–7 paid days per year at full wage replacement, with leave explicitly allowed for family caregiving including attending children's medical appointments (Pichler and Ziebarth, 2024; Mitchell, 2024). Because access to PSL remains highly unequal—with low-wage, part-time, and service-sector workers least likely to have coverage (Bureau of Labor Statistics, 2024)—these mandates disproportionately expand benefits among families facing the steepest time and financial constraints in accessing adolescent mental health care.

This paper provides the first causal evidence, to our knowledge, on the effects of state paid sick leave mandates on adolescents' mental health outcomes. We link the timing and location of paid sick leave (PSL) adoption to child-level survey data from the National Survey of Children's Health (NSCH) and the Youth Risk Behavior Surveillance System (YRBSS) to examine impacts on adolescents' engagement with mental health care and mental health status. Specifically, we study effects on (1) receipt of any mental or behavioral health care,

(2) use of mental health medication, (3) parent-reported depression and anxiety diagnoses, and (4) self-reported depressive symptoms and suicidal ideation. Our empirical approach exploits the staggered adoption of PSL mandates across states and employs the interaction-weighted event-study estimator of Sun and Abraham (2021), which is robust to treatment effect heterogeneity and allows us to assess pre-trends and dynamic treatment effects relative to adoption.

Our findings indicate that PSL mandates substantially increase adolescents' mental health care utilization and clinical identification of mental health conditions. NSCH estimates show that adoption of a PSL mandate increases the probability that an adolescent received any mental or behavioral health care in the past year by 2.3 percentage points, corresponding to a 13.6% increase relative to the pre-policy mean of 17.1%. Depression diagnoses increase by 1.5 percentage points, representing a 14.1% increase relative to the baseline mean of 10.9%. Because these NSCH measures reflect clinical diagnoses and treatment, the observed increases could reflect either improved detection or worsening mental health. Complementary analysis of YRBSS data allows us to discriminate between these interpretations. We find that PSL mandates reduce self-reported depressive symptoms by 2.2 percentage points and passive suicidal ideation by 1.42 percentage points, relative to pre-policy means of 30.3% and 16.5%, respectively, with no detectable effects on active suicidal ideation. The simultaneous increase in formal diagnoses alongside declines in self-reported symptoms is consistent with a detection-and-treatment channel—in which PSL enables earlier identification and effective care—rather than a deterioration in underlying adolescent mental health.

Mechanism analyses provide further support for a time-flexibility interpretation. Following PSL adoption, the probability that a parent reduced or stopped work to care for the adolescent increases by 1.5 percentage points, providing direct evidence that mandates expand parental time flexibility. This increased flexibility facilitates entry into the mental health care system: the 2.3 percentage-point increase in realized care receipt substantially exceeds a modest 0.6 percentage-point increase in reported unmet mental health need, indicating that while some newly aware families encounter supply-side barriers, the dominant effect is expanded service utilization. Increases in reported difficulty coordinating care and obtaining appointments are consistent with newly entering families encountering frictions inherent in a supply-constrained system, rather than a worsening of access conditions.

Our study complements recent evidence on PSL mandates and adult mental health care. Eisenberg et al. (2025) show that PSL mandates do not increase overall mental health care use among working-age adults but instead shift treatment toward more time-intensive modalities such as psychotherapy and inpatient care, with suggestive improvements in self-reported mental health. The distinction between adult and adolescent mental health care access is theoretically important and motivates a separate investigation. Adults make autonomous decisions about their own care: they can schedule appointments, choose providers, and attend sessions independently, so that PSL primarily affects treatment intensity and modality. Adolescents, by contrast, depend on parents for every stage of care access—scheduling, insurance coordination, legal consent, transportation, and often physical presence during sessions. This structural dependence means that parental time constraints can prevent care from being accessed at all, not merely shift its composition. PSL should therefore operate on a different margin for adolescents than for adults: the extensive margin of whether care occurs, rather than the intensive margin of how it is delivered. Moreover, by combining clinical diagnoses,

parental reports, and youths' self-reported symptoms, we distinguish increased detection from changes in underlying mental health and identify an intergenerational spillover channel through which labor policies targeting workers generate mental health benefits for their children.

Against this backdrop, our study contributes to three strands of literature. First, it extends prior research on the health effects of labor market policies (Chen et al., 2020; Guo, 2025; Pichler and Ziebarth, 2017; Callison and Pesko, 2022) by providing the first causal evidence on the impact of paid sick leave mandates on adolescent mental health outcomes. Second, it advances understanding of the intergenerational and spillover consequences of family-oriented labor protections (Maclean and Pabilonia, 2025; Deza et al., 2025; Dahl et al., 2016; Baker and Milligan, 2008), demonstrating that policies designed to support working parents can yield significant benefits for adolescents' mental health and development. Finally, it informs ongoing policy debates on how employment regulations can function as upstream determinants of population mental health (Reeves et al., 2017; Wang et al., 2024). By easing time constraints faced by working families, relatively low-cost PSL mandates appear to deliver meaningful improvements in child and adolescent mental health, underscoring the broader public health potential of labor-based social policy.

Section 2 reviews U.S. paid sick leave policy and outlines the conceptual framework. Section 3 describes the data sources. Section 4 presents the empirical strategy. Section 5 reports results on mental health care utilization, diagnoses, and mental health outcomes. Section 6 concludes with a discussion of implications.

2 Institutional Background

2.1 Adolescent Mental Health Care in the United States

Adolescent mental health care in the United States spans multiple treatment settings with varying time requirements. Primary care physicians provide initial mental health screening, brief counseling, and medication initiation (Zuckerbrot et al., 2018; Cheung et al., 2018). More intensive outpatient mental health care typically includes comprehensive diagnostic evaluation, individual psychotherapy, and medication management delivered by specialty mental health providers. Outpatient mental health services are provided by psychiatrists and other mental health professionals and commonly involve psychotherapy and psychotropic medication use (Olfson et al., 2019; Centers for Medicare & Medicaid Services, 2025). Standard psychotherapy sessions typically last 45–60 minutes (Walkup et al., 2008; Weisz et al., 2017), with evidence-based treatments such as cognitive behavioral therapy generally requiring weekly sessions over 8–20 weeks (March et al., 2007; Weisz et al., 2017).

A defining feature of adolescent mental health care is the legal requirement for parental involvement in most states. Eighteen states either prohibit independent adolescent consent to mental health treatment or provide no explicit authorization for minors to consent independently (Schleider et al., 2025). Parents must schedule appointments, coordinate with insurance providers, arrange transportation, and often attend family therapy sessions or medication consultations. These logistical requirements create structural dependence on parental time availability that distinguishes adolescent from adult mental health care access.

Provider shortages compound these time barriers: approximately 70% of U.S. counties lack a single child or adolescent psychiatrist (McBain et al., 2019), and simulated-patient studies find that fewer than 17% of child psychiatrists have available appointments for new patients (Cama et al., 2017). Consequently, even when families have insurance coverage and motivation to seek care, time constraints—including inflexible work schedules and lengthy wait times—can delay treatment initiation and reduce treatment adherence.

These structural features of adolescent mental health care delivery create a framework in which parental time availability functions as a critical enabling factor for treatment access. Evidence demonstrates that parental access to paid sick leave significantly increases children's healthcare utilization, including preventive care and routine appointments, while reducing reliance on emergency services (Asfaw and Colopy, 2017; Seixas and Macinko, 2020). Although typical PSL mandates provide 3–7 paid days per year—insufficient to cover a full course of weekly psychotherapy—even a modest expansion in protected time may be consequential at the margin. For many families, the binding constraint is not the total number of appointments but the inability to attend even an initial evaluation or a small number of critical sessions without risking income loss or job security. PSL may therefore operate primarily by lowering the threshold for treatment initiation: enabling a first diagnostic appointment, a medication consultation, or enough early sessions to establish a therapeutic relationship and demonstrate benefit, after which families may find ways to sustain engagement through scheduling flexibility, telehealth, or school-based services. In addition, PSL accruals are renewable—workers accumulate new hours each pay period—and both parents in two-parent households can draw on their respective leave balances. State paid sick leave mandates, which provide workers with protected, compensated leave that can be used for family healthcare needs, represent a policy intervention that could directly address these time-related barriers at the critical margin of treatment initiation and early engagement.

2.2 Paid Sick Leave Policy in the United States

The United States does not have a permanent federal paid sick leave mandate, making it an outlier among high-income countries. The Family and Medical Leave Act of 1993 provides up to 12 weeks of unpaid leave for eligible workers, but eligibility is restricted by firm size, job tenure, and hours worked, and the policy does not cover short-term needs such as attending medical appointments (Appelbaum, 2014; Pichler and Ziebarth, 2024). During the COVID-19 pandemic, the Families First Coronavirus Response Act provided temporary paid leave from April to December 2020, but this policy expired without establishing a permanent national standard (Andersen et al., 2023).

In the absence of federal action, paid sick leave policy has developed through state and local legislation. San Francisco enacted the first employer mandate in 2007, followed by the District of Columbia in 2008 and Connecticut in 2012 (Ober, 2006; Gilliam and Ben-Ishai, 2012). As of 2024, 18 states and DC have enacted statewide PSL mandates (Mitchell, 2024). These mandates share common structural features: employees typically accrue paid sick leave at a rate of one hour per 30–40 hours worked, with annual caps of 24–56 hours (3–7 days) compensated at 100% wage replacement (U.S. Department of Labor, 2023; Paycor, Inc., 2025). Critically, leave can be used not only for employees' own health needs but also for family caregiving, with all state mandates explicitly including care for children such

as attending medical appointments or providing care during illness. Mandates include job protection and anti-retaliation provisions and limit employers' ability to require extensive documentation (Pichler and Ziebarth, 2017).

The empirical literature on PSL mandates establishes three broad patterns. First, mandates produce meaningful first-stage effects on leave access and use: employer-provided PSL coverage and worker utilization of sick days increase substantially following adoption (Ahn and Yelowitz, 2016; Callison and Pesko, 2022), with no detectable increase in business bankruptcies and a significant reduction in consumer bankruptcy filings (Miller, 2022). Second, mandates shift health care utilization toward planned, preventive services—increasing prescriptions, vaccinations, and cancer screenings while reducing potentially unnecessary emergency department visits—consistent with workers substituting away from acute care when protected time becomes available (Ma et al., 2022; Callison et al., 2023; Maclean et al., 2024). These utilization gains are accompanied by improvements in economic security, including higher wages for women and reduced poverty rates (Slopen, 2023, 2024). Third, and most relevant for the present study, mandates generate intergenerational spillovers: parents increase primary childcare time by 6%, child maltreatment reports decline by 8%, and adult mental health treatment shifts toward more time-intensive modalities such as psychotherapy (Maclean and Pabilonia, 2025; Deza et al., 2025; Eisenberg et al., 2025). However, existing work focuses almost exclusively on adults' own health care use or on broad child welfare indicators, leaving open whether parental time flexibility translates into improvements in adolescent mental health outcomes that require sustained parental mediation—scheduling, consent, transportation, and appointment attendance.

Access to PSL remains highly stratified: low-wage, part-time, and service-sector workers are substantially less likely to receive employer-provided PSL absent mandates (Bureau of Labor Statistics, 2024). These same populations face elevated barriers to accessing adolescent mental health care (The Pew Charitable Trusts, 2024). Consequently, state PSL mandates disproportionately expand coverage among families for whom time constraints are most binding.

These institutional features of PSL mandates have several implications for adolescent mental health. First, by providing parents with protected time to attend appointments, PSL mandates may increase detection and diagnosis of mental health conditions that were previously unidentified, leading to increases in parent-reported diagnoses and treatment receipt even if underlying mental health remains unchanged. Second, PSL may improve access to evidence-based mental health care by enabling parents to attend initial consultations, diagnostic evaluations, and early therapy sessions that are critical for establishing a treatment plan. Even a small number of additional appointments can be clinically meaningful: evidence from stepped-care models and brief interventions demonstrates that early engagement with mental health services—including initial assessment, psychoeducation, and the first several therapy sessions—can yield significant symptom improvement and reduce suicidal ideation, particularly for adolescents who would otherwise receive no care at all (Kaczkurkin and Foa, 2015; Keles and Idsoe, 2018; van Ballegooijen et al., 2025). Third, PSL mandates may affect adolescent mental health indirectly through improvements in parental mental health (Eisenberg et al., 2025), reduced financial stress (Miller, 2022; Slopen, 2024), and increased parental caregiving time (Maclean and Pabilonia, 2025).

These channels generate distinct and potentially offsetting empirical predictions. In-

creased detection predicts higher rates of parent-reported diagnoses and treatment receipt, while improved access to effective treatment predicts reductions in adolescent self-reported symptoms and suicidal ideation. Observing both patterns simultaneously—increased formal diagnoses alongside reduced self-reported symptoms—would support an interpretation in which PSL mandates ease time-related access barriers and improve both detection and actual mental health outcomes.

3 Data

3.1 Paid Sick Leave Policy Data

We begin by constructing a state–year panel of Paid Sick Leave (PSL) mandates, which serves as the primary policy variable in this analysis. Policy data were drawn from the National Partnership for Women and Families' (2023) compilation of Paid Sick Days Statutes, supplemented with legislative texts and state labor department releases to verify effective dates and coverage provisions. Each law specifies key implementation features, including accrual rates (typically one hour of paid leave for every 30–40 hours worked), annual caps ranging from 24 to 56 hours (3–7 days per year), and eligible uses such as personal illness, family caregiving, or school closures. We coded a binary indicator for whether a state had an active PSL mandate in effect during a given survey year.[1] The final panel includes the 14 states and the District of Columbia that adopted mandates during our study period, 2012–2022 (Table 1).

(Table 1 here)

3.2 National Survey of Children's Health (NSCH) Data and Outcomes

Our primary data source is the National Survey of Children's Health (NSCH), a nationally representative cross-sectional survey administered annually by the U.S. Census Bureau on behalf of the Health Resources and Services Administration's Maternal and Child Health Bureau. The NSCH collects detailed information on children's physical, mental, and behavioral health, health care access, family environment, and social determinants of well-being. The survey employs a stratified address-based sampling design and is completed online or by mail by a parent or guardian who is knowledgeable about the child's health and development. Each year, the NSCH includes approximately 35,000 to 45,000 completed child-level interviews, representing the noninstitutionalized population of children aged 0–17 years in all 50 states and the District of Columbia (U.S. Census Bureau, 2023).

Our analysis uses pooled data from the 2016–2022 NSCH waves, corresponding to the period following the NSCH redesign that standardized sampling procedures, weighting, and questionnaire content across years. We begin the study period in 2016 to ensure consistency

[1] Because NSCH and YRBSS collect information throughout the calendar year, states with laws taking effect after July were coded as treated beginning in the following year to ensure consistent alignment between policy exposure and the survey reference period.

in outcome measurement and survey design and end in 2022, the most recent wave available at the time of analysis. The analytic sample is restricted to adolescents aged 12–17, the age range for which mental health service use, medication use, and diagnostic indicators are most reliably and consistently reported in the NSCH. After applying these restrictions and excluding observations with missing key covariates, the final analytic sample includes 93,875 adolescent observations.

The unit of analysis is an individual adolescent observed in a given survey year. We use publicly available state identifiers to merge adolescent-level outcomes from the NSCH with a state–year panel capturing the timing of paid sick leave (PSL) mandate adoption and other time-varying state policy and economic conditions. Our primary outcomes are constructed as binary indicators equal to one if an adolescent received any mental or behavioral health care in the past 12 months or used medication for emotional or behavioral conditions, capturing engagement with clinical treatment, and zero otherwise. We additionally examine binary indicators for parent-reported diagnoses of depression, anxiety, and attention-deficit/hyperactivity disorder (ADHD), which reflect clinical identification and diagnosis rather than self-reported symptoms. Together, these measures capture distinct stages of the mental health care pathway, ranging from service utilization to diagnostic recognition.

Importantly, the NSCH mental health diagnosis variables capture parent-reported clinical diagnoses made by a health professional, rather than symptom scales. Parents are asked whether a doctor or other health care provider has ever told them that their child has depression, anxiety, or ADHD. This distinction is central for interpretation, as changes in diagnoses reflect access to care and clinical identification rather than underlying symptom prevalence.

Table 2 reports full-sample summary statistics for the analytic sample pooled across survey years and separately for states with and without a PSL mandate in effect during the survey year. In the full sample, 17.6% of adolescents received any mental health care in the past 12 months, 15.8% used medication for emotional or behavioral conditions, and 11.0%, 18.0%, and 15.4% had parent-reported diagnoses of depression, anxiety, and ADHD, respectively. Adolescents residing in PSL mandate states are descriptively more likely to receive any mental health care, 19.0% versus 17.2%, and to have anxiety diagnoses, 19.3% versus 17.7%, compared with adolescents in non-mandate states. In contrast, rates of medication use and ADHD diagnoses are modestly lower in PSL mandate states.

The demographic composition of adolescents also differs across states with and without PSL mandates. Adolescents in mandate states are slightly more likely to reside in households with higher parental educational attainment, with 63.9% having a parent with a college degree or higher compared with 56.9% in non-mandate states. Racial and ethnic composition varies across groups, with PSL mandate states having a lower share of White, non-Hispanic adolescents and a higher share of Hispanic adolescents. Insurance coverage is high in both groups, though slightly higher in mandate states, 95.4% versus 94.4%. At the state level, PSL mandate states tend to exhibit higher minimum wages, broader Medicaid eligibility thresholds for both parents and non-parents, higher housing prices, and greater primary care physician availability, alongside more stringent COVID-19 policy responses. These patterns underscore systematic differences in policy and economic environments across states that motivate the inclusion of rich state-level controls in the regression analysis.

(Table 2 here)

3.3 Youth Risk Behavior Surveillance System (YRBSS) Data and Outcomes

The data on self-reported depression and suicidal ideation among youth are taken from the Youth Risk Behavior Surveillance System (YRBSS), a biennial survey conducted by the U.S. Centers for Disease Control and Prevention (CDC) since 1991. The YRBSS High School Survey is administered to students in grades 9–12, typically aged 12 to 18 years, and is designed to be representative at the state level in participating states for monitoring youth health, safety, and risk behaviors. The YRBSS employs a two-stage cluster sampling design, sampling schools in the first stage and classrooms within schools in the second stage. Survey weights are provided to account for differential selection probabilities and nonresponse. We pooled nine biennial YRBSS waves from 2005 through 2021, covering both the pre- and post-policy periods relevant for our analysis.

We used three distinct measures of mental health derived from YRBSS questions. The first is the self-reported depression, which is captured by the youth response to the question: "During the past 12 months, did you ever feel so sad or hopeless almost every day for two weeks or more in a row that you stopped doing some usual activities?". The question aligns closely with established diagnostic criteria for a depressive episode and widely used as a measure of depressive symptoms in population-based research. The second is the passive suicidal ideation which measures the binary response if the individual seriously considered attempting suicide in the last twelve months. Lastly, the third outcome is active suicidal ideation, measured by a binary indicator for whether the individual reports having made a suicide plan in the past 12 months. Table A.1 reports summary statistics for these outcomes. Approximately 30 percent of students report depressive symptoms, 16.8 percent report suicidal ideation, and 13.7 percent report having made a suicide plan. In contrast to the NSCH, YRBSS outcomes capture adolescents' self-reported emotional states and behaviors rather than clinical diagnoses. For example, the depressive symptoms measure reflects whether respondents experienced prolonged feelings of sadness or hopelessness that interfered with daily activities, aligning with symptom-based screening rather than formal diagnosis.

These three mental health measures were selected because the phrasing of these questions has remained consistent across all survey waves dating back to 2005. This consistency is critical for our research design, as it provides a pre-intervention period prior to the implementation of Connecticut's state-level Paid Sick Leave (PSL) mandate in 2012. Although additional mental health items were introduced in more recent YRBSS surveys, we limit our analysis to these three outcomes because they are the only ones with sufficient pre-policy coverage. In addition to these outcome variables, we incorporate individual-level covariates from the survey—specifically age, sex, and race/ethnicity—which we also use to examine heterogeneous treatment effects.

4 Empirical Strategy

We estimate the causal effect of state paid sick leave (PSL) mandates on mental health outcomes using variation in the timing of PSL adoption across U.S. states. States adopt mandates in different years (Table 1), and once adopted, mandates remain in effect. This staggered rollout creates a difference-in-differences design with multiple treatment cohorts. In settings with staggered treatment adoption, conventional two-way fixed effects (TWFE) estimators can be problematic. When treatment effects are heterogeneous across cohorts or evolve over time, TWFE implicitly compares early-treated units to later-treated units, using already-treated observations as controls. These comparisons can receive negative or non-convex weights, leading to estimates that are difficult to interpret and potentially biased relative to any meaningful average treatment effect (Goodman-Bacon, 2021). Therefore, we use the interaction-weighted event-study estimator of Sun and Abraham (2021), which is robust to staggered adoption and heterogeneous treatment effects. The Sun–Abraham estimator constructs treatment effects by comparing each treated cohort only to states that are not yet treated or never treated at the same point in time. The coefficients θ_{gk} represent cohort- and event-time–specific average treatment effects, measuring the effect of PSL adoption k years relative to implementation for states first treated in year g. Aggregate treatment effects are then weighted averages of these cohort-specific estimates, with weights that are non-negative and sum to one. As a result, both the point estimates and the event-study coefficients have a clear causal interpretation as averages of well-defined cohort-level treatment effects, free from contamination by already-treated units.

$$Y_{ist} = \alpha_s + \lambda_t + X'_{st}\beta + I'_{ist}\omega + \sum_{g \in \mathcal{G}} \sum_{\substack{k \in \mathcal{K} \\ k \neq -1}} \theta_{gk}\, \mathbf{1}\{G_s = g\}\mathbf{1}\{t - g = k\} + \varepsilon_{ist}. \tag{1}$$

Here, Y_{ist} is the outcome for individual i in state s and year t; G_s denotes the first year a PSL mandate takes effect in state s; and $k = t - g$ indexes event time, with $k = -1$ omitted as the reference period. The specification includes state fixed effects α_s, year fixed effects λ_t, state-level time-varying controls X_{st}, and individual-level controls I_{ist}. In the NSCH, individual-level controls I_{ist} include adolescent age, sex, race/ethnicity, insurance type, household federal poverty level, parental education, household language, number of adults and children in the household, and parental marital status. In the YRBSS, individual-level controls include age, sex, and race/ethnicity. State-level time-varying controls X_{st} include the state minimum wage, unemployment rate, Medicaid income eligibility thresholds for parents and non-parents, community health center and primary care physician supply per capita, the Housing Price Index, log per capita income, and COVID-19 cumulative case and death rates with the Oxford Government Response Index. These controls address concurrent economic conditions, health care supply factors, and policy changes that could independently affect both PSL adoption timing and adolescent mental health outcomes.[2] We estimate Equation (1) using survey sampling weights to ensure representativeness while clustering standard errors at the state level to account for policy variation and serial correlation. Never-treated states serve as the comparison group. The estimator recovers cohort-specific dynamic

[2]Because the YRBSS is administered biennially, state-level controls in the YRBSS specifications are averaged over the two-year survey period to align with the survey reference window.

treatment effects and aggregates them using non-negative weights, avoiding contamination from comparisons across differently treated cohorts.

As required by the Sun–Abraham estimator, we exclude cohorts that are always treated throughout the sample period. In the NSCH (2016–2022), states that adopted PSL mandates prior to or in the first year of the panel—Connecticut (2012), the District of Columbia (2008), California (2015), Massachusetts (2015), and Oregon (2016)—are excluded from estimation because they lack any pre-treatment observations. The remaining treated cohorts are: Vermont (treated 2017; 1 pre-treatment year), Arizona, Maryland, and Washington (treated 2018; 2 pre-treatment years), New Jersey and Rhode Island (treated 2019; 3 pre-treatment years), and New York and Colorado (treated 2021; 5 pre-treatment years). Three of the four treatment cohorts thus have at least two pre-treatment periods, permitting meaningful assessment of pre-trends for the majority of identifying variation. In the YRBSS, which extends back to 2005, all treatment cohorts—including early adopters—contribute multiple pre-treatment periods, providing a stronger basis for evaluating the parallel trends assumption.

The identification relies on the parallel trends assumption: absent PSL adoption, treated and comparison states would have followed similar trends in mental health outcomes. To assess the plausibility of this assumption, we implement the estimator of Sun and Abraham (2021) in an event-study framework with multiple pre-treatment and post-treatment periods relative to adoption. Pre-treatment leads are used to test for differential pre-trends, while post-treatment lags trace the dynamic evolution of treatment effects. Event-time coefficients are interpreted relative to the year immediately preceding adoption.

5 Results

5.1 Adolescent Mental Health Outcomes from the NSCH

Table 3 reports estimates of the effect of state paid sick leave (PSL) mandates on adolescent mental health care utilization and diagnostic outcomes, estimated using Equation (1). The results indicate that PSL mandates are associated with statistically significant increases in adolescents' engagement with mental health care. Following adoption of a PSL mandate, the probability that an adolescent received any mental or behavioral health care in the past 12 months increases by 2.3 percentage points ($p < 0.05$), corresponding to a 13.6% increase relative to the pre-policy mean of 17.1%. We also find a positive increase in medication use for emotional or behavioral conditions of 1.4 percentage points, which is statistically significant at the 10% level and represents an 8.6% increase relative to its baseline mean of 15.9%.

(Table 3 here)

PSL mandates are associated with higher rates of parent-reported mental health diagnoses among adolescents. In particular, the probability of a depression diagnosis increases by 1.5 percentage points ($p < 0.05$), corresponding to a 14.1% increase relative to the baseline prevalence of 10.9%. We also estimate a positive effect on anxiety diagnoses of 1.2 percentage points that is marginally statistically significant at the 10% level, representing a 7.0%

increase relative to the baseline mean of 17.5%. The largest and most precisely estimated effects are observed for outcomes related to mental health care utilization and depression diagnoses, and these patterns remain stable across alternative specifications that sequentially add demographic, economic, health care supply, and COVID-related controls, as shown in Table A.2.

Figure 1 reports event-study estimates for receipt of any mental health care and parent-reported depression diagnoses. The coefficients on the pre-treatment leads are small in magnitude and statistically indistinguishable from zero, providing no evidence of differential pre-trends between adopting and non-adopting states prior to policy implementation. Beginning in the first year following adoption, we observe positive and statistically significant increases in adolescents' receipt of mental health care, with effects that persist and modestly grow over subsequent years. For depression diagnoses, post-adoption estimates are generally positive and often marginally statistically significant, suggesting modest increases following PSL adoption, though estimates are less precisely estimated than those for mental health care utilization. Event-study estimates for anxiety diagnoses do not satisfy the parallel trends assumption and are therefore not interpreted dynamically. Estimates for medication use for emotional or behavioral conditions are noisy and do not exhibit a clear or stable post-adoption trajectory.

(Figure 1 here)

Table 5 presents heterogeneous effects of paid sick leave mandates across adolescent and family characteristics. For receipt of any mental health care, statistically significant increases are concentrated among younger adolescents aged 12–14, White adolescents, and females, with positive but imprecise estimates for other groups. In contrast, effects on depression diagnoses are concentrated among older adolescents aged 15–17, White, Asian, and Black adolescents, females, and adolescents with college-educated parents. Estimates for males and for several racial and ethnic subgroups are smaller and not statistically distinguishable from zero. Overall, the subgroup results indicate that increases in mental health care utilization and diagnostic identification are not uniform across populations, with distinct patterns by age, sex, race and ethnicity, and parental education.

(Table 5 here)

Mental health conditions and diagnostic identification vary meaningfully across adolescent developmental stages, with older adolescents exhibiting higher prevalence of depressive and anxiety disorders, greater autonomy in seeking care, and more frequent contact with school-based mental health services. Consistent with these patterns, estimated effects on depression diagnoses are concentrated among adolescents aged 15–17, whereas increases in general mental health care use are observed among both younger and older adolescents. These patterns are consistent with known developmental gradients in symptom onset, recognition, and care engagement rather than differences in economic or labor market exposure.

Taken together, the NSCH results reflect increased engagement with the mental health care system and greater diagnostic identification following PSL adoption. Because these measures capture diagnosed conditions rather than symptoms, the observed increases could reflect either improved detection of previously undiagnosed conditions or a genuine worsening of underlying adolescent mental health.

5.2 Youth Mental Health Outcomes from the YRBSS

In contrast to the diagnosis-based outcomes in the NSCH, the YRBSS captures self-reported symptoms, allowing us to examine whether increased detection and treatment correspond to changes in adolescents' emotional well-being. Table 4 reports estimates from the Sun and Abraham (2021) interaction-weighted event-study specification described in Equation (1).

We find statistically significant negative effects of PSL mandates on reported depression and passive suicidal ideation. The estimated coefficient for depression implies a 2.2 percentage-point decline ($p < 0.05$), corresponding to a 7.2% reduction relative to a pre-treatment mean of 30.3%. Passive suicidal ideation decreases by 1.42 percentage points ($p < 0.01$), representing an 8.6% decline relative to a baseline mean of 16.5%. In contrast, the estimated effect on active suicidal ideation is not statistically significant at conventional levels ($p = 0.102$), although the point estimate is negative at 1.21 percentage points (9.0%) relative to a pre-treatment mean of 13.5%.

(Table 4 here)

Figure 2 presents interaction-weighted event-study estimates of the effects of paid sick leave mandates on youth mental health outcomes, based on Equation (1). Coefficients are normalized relative to the year immediately preceding adoption ($t = -1$). Across all three reported mental health outcomes, the estimated pre-treatment coefficients are small in magnitude and statistically indistinguishable from zero, with no systematic patterns prior to adoption. This provides no evidence of differential pre-trends and supports the validity of the parallel trends assumption underlying our identification strategy.

Following adoption, the post-treatment dynamics differ across outcomes. For reported depression, the point estimates turn negative after implementation and remain predominantly below zero across most post-adoption periods, with larger declines emerging in later years, although precision varies across event times. A similar pattern is observed for passive suicidal ideation, where estimates become negative shortly after adoption and remain generally below zero over subsequent periods, consistent with a sustained decline, although with widening confidence intervals at longer horizons. In contrast, estimates for active suicidal ideation are substantially more imprecise, with post-adoption coefficients that are generally negative in sign but largely statistically insignificant.

(Figure 2 here)

The aggregate estimates of health measures may mask significant heterogeneity across demographic subgroups. To examine potential heterogeneity in these effects, Table 6 reports subgroup analyses by age, sex, and race/ethnicity. For reported depression (Panel A), the estimated effects are negative and statistically significant for both males and females, with larger magnitudes for females. By age, the decrease in reported depression is concentrated among youth aged 15 or older, while estimates for younger adolescents are small and statistically insignificant. Across racial groups, statistically significant reductions are observed for White and Black youth with the larger magnitude for black youth whereas estimates for Asian, Hispanic, and other racial groups are imprecisely estimated and not statistically

significant. Panel B presents results for passive suicidal ideation and are similar to the reported depression. We find statistically significant reductions for both females and males, with larger effects for females. By race, the estimates indicate significant declines for White and Black youth, while effects for Asian, Hispanic, and other racial groups are not precisely estimated. However, unlike depression, reductions in passive suicidal ideation are evident for both younger adolescents and those aged 15 or older. Finally, Panel C reports estimates for active suicidal ideation. The effects are generally smaller in magnitude and less precisely estimated across subgroups. While some subgroups—such as females, youth aged 15 or older, and Black youth—exhibit negative and statistically significant point estimates, the majority of subgroup estimates are statistically indistinguishable from zero. Overall, the subgroup analysis suggests that the mental health benefits of paid sick leave mandates are concentrated among older adolescents and certain demographic groups.

(Table 6 here)

5.3 Potential Mechanism

State paid sick leave (PSL) mandates are unlikely to directly affect adolescents' mental health outcomes. Instead, these mandates may operate through changes in parents' ability to adjust work responsibilities, navigate the health care system, and allocate time to caregiving. In this section, we trace a pipeline from parental time flexibility to care engagement to system-level frictions, examining how each stage links PSL mandates to adolescent mental health outcomes.

The most direct mechanism operates through parental time allocation and work adjustment. Table 7 reports the effects of PSL mandates on parental labor supply and family interaction. We find that PSL adoption increases the probability that a parent reduced or stopped work to care for the adolescent by 1.5 percentage points (27.9%), relative to a baseline mean of 5.3%. This result provides direct evidence that PSL mandates expand parents' capacity to temporarily adjust work responsibilities when adolescents require care. In contrast, estimated effects on family meals, parent–adolescent communication, and collaborative problem solving are positive but imprecisely estimated, suggesting that the additional time flexibility is channeled toward specific caregiving demands rather than diffusely improving general family routines (Maclean and Pabilonia, 2025; Deza et al., 2025).

(Table 7 here)

This increased parental time flexibility, in turn, facilitates greater engagement with the mental health care system. As documented in Section 5.1, PSL mandates increase receipt of any mental health care by 2.3 percentage points and depression diagnoses by 1.5 percentage points, indicating that more families are entering the care system following adoption.

A natural consequence of expanded utilization in a supply-constrained system is that newly entering families encounter frictions that were previously irrelevant to them. Table 8 presents evidence on care coordination and access barriers. PSL adoption is associated with a 0.6 percentage-point increase in reported unmet mental health need, relative to a baseline prevalence of 1.3%. Critically, this variable is measured unconditionally: all parents

are asked whether their child needed mental health care but did not receive it, with those reporting no need coded as zero. The observed increase therefore reflects a rise in the number of families who *recognize* a mental health need—likely as a result of increased contact with health professionals following PSL adoption—but have not yet completed the full treatment pathway. We also observe a 2.1 percentage-point increase in reported difficulty coordinating care (12.1%, relative to a baseline mean of 17.4%) and a 1.1 percentage-point increase in trouble obtaining an appointment (58.4%, relative to a baseline mean of 1.9%). Importantly, these access barriers should be interpreted in light of the main results: the 2.3 percentage-point increase in realized mental health care receipt is nearly four times the magnitude of the 0.6 percentage-point increase in reported unmet need, indicating that while a small share of newly aware families encounter supply-side barriers, the dominant effect of PSL mandates is to increase realized service utilization. Estimated effects on the need for home-based care, facility hours not aligning with need, avoidance of changing providers, and receipt of a routine preventive check-up are smaller in magnitude and not statistically distinguishable from zero (Mitchell and Selmes, 2007; Hansen et al., 2021; Chen et al., 2020).

(Table 8 here)

Taken together, the mechanism results trace a coherent pipeline: PSL mandates first expand parents' capacity to adjust work schedules (increased work stoppage/reduction), which facilitates entry into the mental health care system (increased service receipt and diagnoses), which in turn surfaces previously unrecognized needs among some families (increased awareness of unmet need) and exposes newly entering families to the coordination frictions inherent in a supply-constrained mental health care system (increased difficulty with appointments and care coordination). The net effect across this pipeline is strongly positive: realized care utilization increases substantially, and—as shown in the YRBSS analysis—self-reported symptoms decline, consistent with effective treatment following expanded access.

5.4 Robustness and Validation Analyses

We conduct a series of robustness and validation exercises to assess the sensitivity of our main findings to alternative specifications, outcome definitions, and sample restrictions. First, we re-estimate the baseline models using a conventional two-way fixed effects (TWFE) difference-in-differences specification. Tables A.4 and A.5 report TWFE estimates for adolescent mental health care utilization and diagnoses in the National Survey of Children's Health (NSCH), and for youth-reported depression and suicidal ideation in the Youth Risk Behavior Surveillance System (YRBSS), respectively. Although TWFE estimators can be biased in settings with staggered policy adoption, the direction and relative magnitude of the TWFE coefficients are consistent with those obtained using the Sun and Abraham (2021) estimator in the main analysis. In addition, Tables A.6 and A.7 re-estimate the Sun and Abraham (2021) models after excluding observations from 2020, a period characterized by substantial disruptions to health care access and mental health. The estimated effects on receipt of any mental health care remain positive and statistically significant at conventional levels ($p = 0.055$), while the coefficient on depression diagnosis remains positive and marginally significant ($p = 0.097$). Estimated effects on anxiety diagnosis and mental health

medication use are attenuated and imprecisely estimated in this restricted sample. Similarly, the estimated effect on reported depression from YRBSS remains negative and statistically significant at conventional levels ($p = 0.013$). The effect on passive suicidal ideation also remains negative but marginally significant ($p = 0.065$). These results indicate that the core findings on mental health care engagement, depression, reported depression and passive suicidal ideation are not driven by pandemic-related shocks, though statistical power is reduced when excluding the COVID year.

We further assess the validity of our results by examining outcomes that should be less directly affected by short-run changes in parental time flexibility or mental health care engagement. Table A.8 reports Sun and Abraham (2021) estimates for diagnostic scope outcomes, including parent-reported ADHD diagnosis, autism diagnosis, and special health care needs (CSHCN). We find limited and imprecise effects across these measures, suggesting that the observed increases in depression and anxiety diagnoses are unlikely to reflect broad diagnostic expansion or changes in reporting of neurodevelopmental conditions. In addition, Table A.9 presents placebo tests for non–time-sensitive child outcomes such as hearing impairment, overweight or obesity status, neighborhood safety, oral health problems, and social skills. Across these outcomes, we find no systematic evidence of policy effects. Together, these robustness checks support the interpretation that paid sick leave mandates increase adolescents' engagement with mental health care and the identification of depression and anxiety, rather than reflecting model-specific artifacts, generalized changes in child health, or spurious correlations. Similarly, Table A.10 presents the placebo test for risk behaviors of youth such as driving in a car with someone who is drunk, driving drunk, and distracted driving from YRBSS. All three risk measures are statistically insignificant, with p-values near one, suggesting the paid sick leave effect on reported depression and suicidal ideation is unlikely to be driven by model specification, sample composition, or spurious correlation.

6 Discussion and Conclusion

This paper provides the first causal evidence that state paid sick leave mandates improve adolescents' mental health outcomes. PSL mandates increase mental health care receipt and depression diagnoses (NSCH) while simultaneously reducing self-reported depressive symptoms and passive suicidal ideation (YRBSS), with effects concentrated among older adolescents, females, and Black youth. This section interprets these findings, benchmarks their magnitude, and discusses limitations.

A central interpretive question is whether the increase in NSCH diagnoses reflects improved detection or a genuine worsening of adolescent mental health. Several lines of evidence favor a detection interpretation. First, and most directly, the YRBSS analysis provides a population-level test: if worsening mental health were driving the NSCH diagnosis increases, self-reported depressive symptoms should also rise in the general student population. Instead, we observe significant declines in both depressive symptoms and passive suicidal ideation, consistent with newly detected adolescents receiving effective treatment. Second, the increase in mental health care receipt (13.6%) is proportionally larger than the increase in medication use (8.6%), suggesting that newly detected cases are disproportionately mild—entering the care system through counseling or evaluation rather than requiring

pharmacological treatment. Third, no effects are observed on ADHD or autism diagnoses (Table A.8), ruling out a generalized diagnostic expansion in which providers systematically diagnose more of all conditions. The simultaneous observation of increased formal diagnoses (NSCH) alongside decreased self-reported symptoms (YRBSS) is difficult to reconcile with a worsening-mental-health hypothesis but follows directly from a detection-and-treatment channel.

The magnitude of these effects is informative when benchmarked against other policies targeting adolescent mental health care. Our estimated 2.3 percentage-point (13.6%) increase in mental health care receipt is comparable to the 2–3 percentage-point gains associated with ACA Medicaid expansions (Ortega, 2023), a far more expensive and comprehensive coverage reform. By contrast, policies that reduce financial barriers without addressing time constraints show limited effects on the extensive margin: federal mental health parity mandates produced no significant increase in the probability of receiving mental health treatment (Drake et al., 2019), and minimum wage increases—which relax income constraints—had no detectable effect on adolescent mental health care utilization (Kavanagh et al., 2024). This pattern suggests that time-based access barriers constitute a distinct and policy-relevant dimension of unmet need, one that financial interventions alone do not resolve. Moreover, the magnitude of PSL effects on care coordination difficulties is consistent with Pennap et al. (2020), who find that disruptions in care continuity increase pediatric emergency department use by 1–2 percentage points—a comparable scale to the coordination frictions we observe among newly entering families.

A natural question is whether 3–7 paid days per year can plausibly produce the observed symptom reductions. Clinical evidence suggests that even modest early engagement yields meaningful gains: meta-analyses of cognitive behavioral therapy for adolescent depression and anxiety find significant treatment effects within the first several sessions (Kaczkurkin and Foa, 2015; Keles and Idsoe, 2018), and a recent systematic review demonstrates that even brief psychotherapy exposure is associated with reductions in suicidal ideation (van Ballegooijen et al., 2025). PSL need not fund a full course of treatment to be consequential; by enabling an initial diagnostic appointment or enough early sessions to establish a therapeutic relationship, it may lower the threshold for treatment initiation, after which families can sustain engagement through scheduling flexibility, telehealth, or school-based services. Moreover, PSL accruals are renewable each pay period and both parents in two-parent households can draw on their respective leave balances, compounding the available protected time over a treatment year.

The findings are also consistent with a time-constraint mechanism: by providing parents with paid, job-protected leave, PSL laws reduce scheduling barriers to care. Our mechanism analyses show that PSL adoption increases the probability that a parent reduced or stopped work to care for the adolescent, while newly entering families report increased difficulty coordinating care and obtaining appointments—patterns consistent with expanded engagement with a supply-constrained mental health care system. These results echo prior work documenting that PSL mandates increase parental caregiving time (Maclean and Pabilonia, 2025), improve family stability (Deza et al., 2025), and raise healthcare utilization among adults (Chen et al., 2020; Guo, 2025). Extending this literature, our findings show that such benefits extend beyond the workplace—facilitating earlier detection, treatment, and prevention of mental health conditions in children and adolescents.

The pattern of null effects across our robustness analyses further sharpens interpretation. PSL mandates produce no detectable effects on ADHD or autism diagnoses, special health care needs, or placebo outcomes such as hearing impairment, obesity, or neighborhood safety. This outcome specificity is theoretically coherent: conditions like ADHD and autism require longer-term behavioral assessments and are typically identified earlier in childhood, making them less responsive to marginal increases in parental time flexibility. Depression, by contrast, often emerges during adolescence and can be identified through a single clinical encounter—precisely the type of time-sensitive service that PSL facilitates. The absence of effects on non–mental-health outcomes rules out the possibility that our estimates reflect broad changes in survey reporting, health care utilization, or state-level trends coinciding with PSL adoption.

The policy implications are substantial. As documented in Section 2, access to paid leave remains highly unequal in the absence of a federal standard, and mandates disproportionately expand coverage among families facing the steepest barriers to care. Expanding PSL coverage could therefore help reduce inequities in child mental health. Moreover, as policymakers seek upstream strategies to address the youth mental health crisis, PSL offers a scalable, low-cost intervention that simultaneously supports labor market stability and public health. Framing PSL as a family health policy—rather than solely a labor protection—may also broaden bipartisan support for its adoption.

Several limitations merit consideration. First, our outcomes rely on parent-reported (NSCH) and self-reported (YRBSS) survey measures, which may be subject to reporting bias; however, combining both data sources provides complementary perspectives that neither alone could offer. Second, the NSCH panel begins in 2016, limiting the number of pre-treatment periods available for some treatment cohorts—particularly Vermont, which contributes only one pre-treatment year. While three of four NSCH treatment cohorts have at least two pre-treatment periods and the YRBSS provides a longer pre-period, limited pre-treatment data may reduce statistical power for detecting pre-trend violations. Finally, state-level policy coding may mask heterogeneity in local implementation, employer compliance, or enforcement intensity. Future research should examine downstream effects on treatment completion, academic outcomes, and broader family well-being, and should leverage administrative claims data to complement the survey-based evidence presented here.

In conclusion, our results highlight that Paid Sick Leave mandates, while designed as worker protections, deliver measurable mental health benefits for children and adolescents. By easing time constraints on working parents, these policies promote earlier care-seeking and emotional well-being, particularly among disadvantaged families. As the United States confronts widening disparities in youth mental health, integrating labor-based policies like PSL into public health and family policy frameworks represents a promising and underutilized avenue for prevention.

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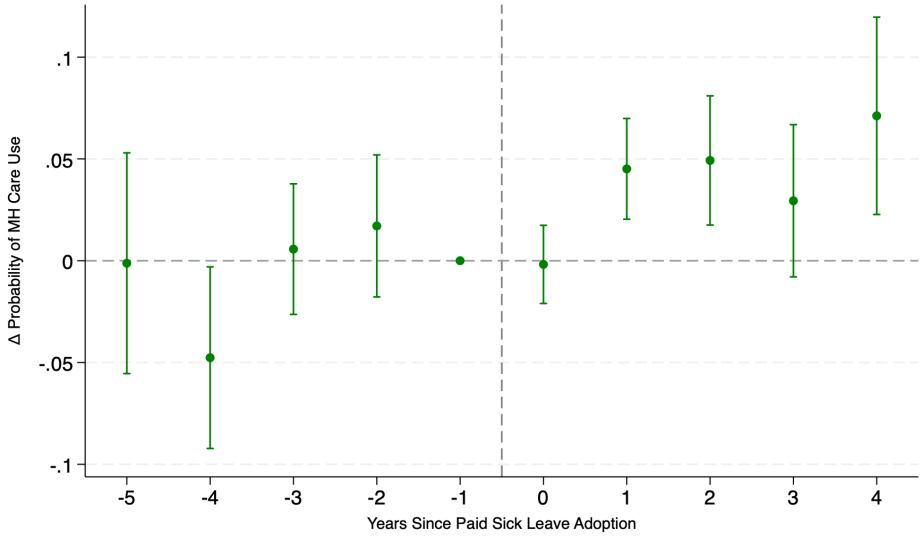
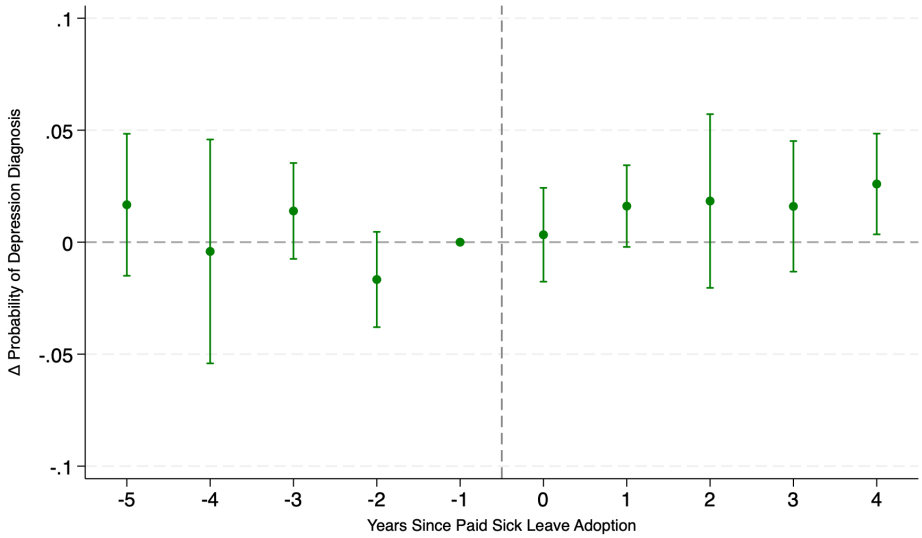
Figure 1: Event-Study Estimates of the Impact of Paid Sick Leave Mandates on Adolescent Mental Health Outcomes (NSCH, 2016–2022)

(a) Received Any Mental Health Care



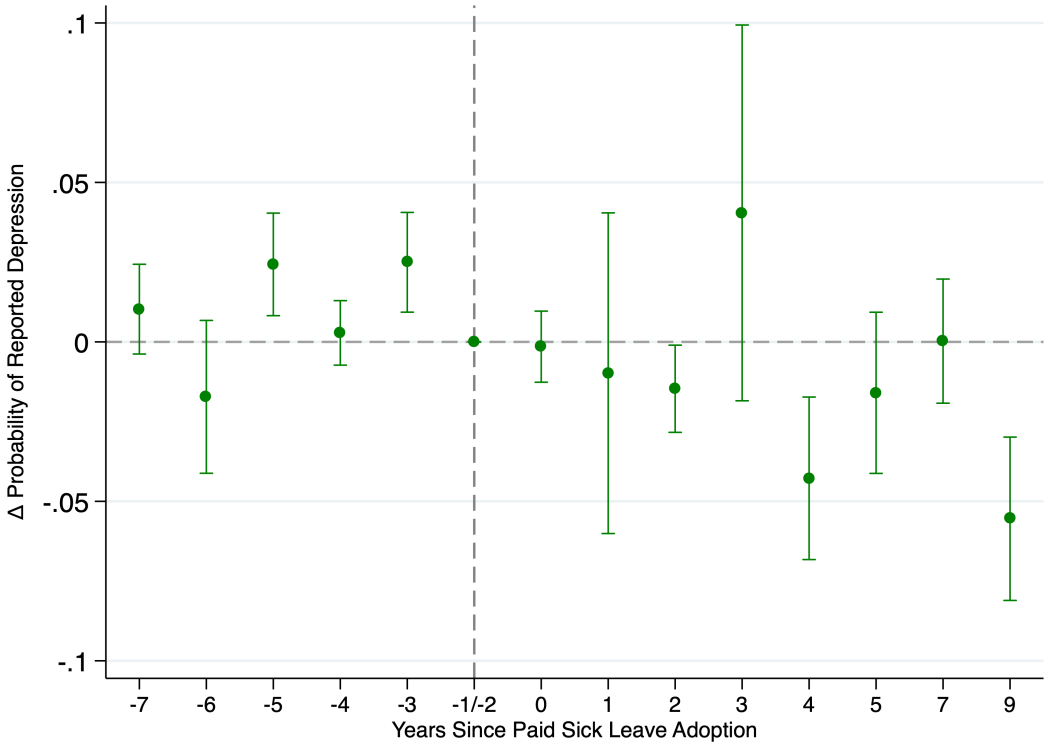
(b) Depression Diagnosis



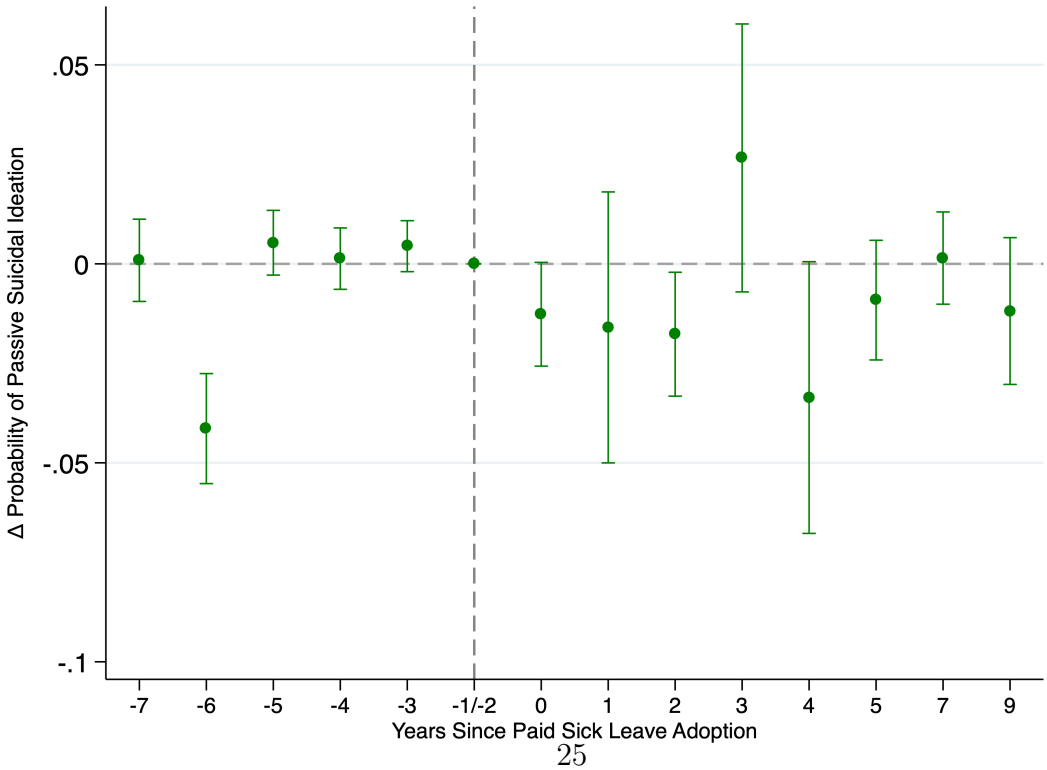
Notes: This figure presents event-study estimates of the effects of state paid sick leave (PSL) mandates on two adolescent mental health outcomes. Panel (a) plots coefficients for receipt of any mental health care, and Panel (b) plots coefficients for parent-reported depression diagnosis. Coefficients are shown relative to the year immediately preceding mandate adoption ($t = -1$), with 95% confidence intervals. All models are estimated using the Sun and Abraham (2021) interaction-weighted difference-in-differences framework, which accounts for staggered policy adoption and heterogeneous treatment effects. Covariates include adolescent age, sex, race/ethnicity, insurance type, household federal poverty level, parental education and marital status, household language, number of adults and children in the household, and state-level controls including the minimum wage, unemployment rate, Medicaid eligibility thresholds, health care provider supply, the Housing Price Index, log per capita income, and COVID-19 case and death rates with the Oxford Government Response Index. All models include state and year fixed effects and are weighted using NSCH survey sampling weights. Standard errors are clustered at the state level.

Figure 2: Event-Study Estimates of the Impact of Paid Sick Leave Mandates on Youth Mental Health (YRBSS, 2005–2021)

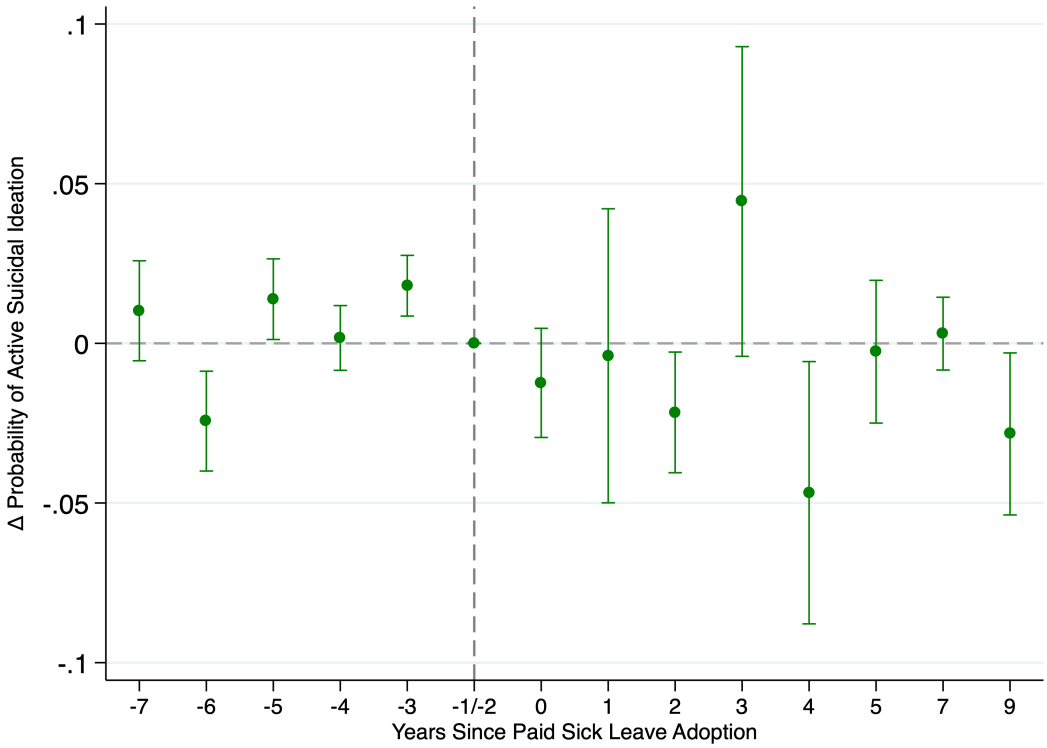
(a) Depression



(b) Passive Suicidal Ideation



(c) Active Suicidal Ideation



Notes: This figure presents event-study estimates of the effects of state paid sick leave (PSL) mandates on three youth self-reported mental health outcomes. Panel (a) plots coefficients for self-reported depression (feeling sad or hopeless for two or more weeks), Panel (b) plots coefficients for passive suicidal ideation (seriously considered attempting suicide), and Panel (c) plots coefficients for active suicidal ideation (made a suicide plan). Coefficients are shown relative to the year immediately preceding mandate adoption ($t = -1$), with 95% confidence intervals. All models are estimated using the Sun and Abraham (2021) interaction-weighted difference-in-differences framework, which accounts for staggered policy adoption and heterogeneous treatment effects. Covariates include age, sex, and race/ethnicity, and state-level controls including the minimum wage, unemployment rate, Medicaid eligibility thresholds, health care provider supply, the Housing Price Index, log per capita income, and COVID-19 case and death rates with the Oxford Government Response Index. All models include state and year fixed effects and are weighted using YRBSS survey sampling weights. Standard errors are clustered at the state level.

Table 1: State's Adoption of Paid Sick Leave Mandate

State	**Effective Date**
Arizona	July 2017
California	July 2015
Colorado	January 2021
Connecticut	January 2012
District of Columbia	November 2008
Maryland	February 2018
Massachusetts	July 2015
New Jersey	October 2018
New Mexico	July 2022
New York	January 2021
Oregon	January 2016
Rhode Island	July 2018
Vermont	January 2017
Washington	January 2018

Source: National Partnership for Women and Families' (2023) compilation of Paid Sick Days Statutes

Table 2: Descriptive Statistics (NSCH 2016–2022)

	Full Sample	**Untreated States**	**Treated States**
Outcomes			
Received Any Mental Health Care	0.176	0.172	0.19
Takes Medication for MH Issues	0.158	0.163	0.139
Depression Diagnosis	0.11	0.111	0.109
Anxiety Diagnosis	0.18	0.177	0.193
Demographics			
Female	0.517	0.518	0.515
Child Age (Years)	14.692	14.695	14.678
Number of Adults in Household	2.228	2.22	2.263
Number of Children in Household	1.705	1.714	1.668
Parent Married	0.73	0.732	0.724
Parent Never Married	0.043	0.042	0.044
Parent Divorced	0.12	0.12	0.119
Parent Separated	0.021	0.02	0.023
Parent Unmarried, Partnered	0.037	0.036	0.04
Parent Widowed	0.021	0.022	0.019
Parental Education: College Degree or Higher	0.583	0.569	0.639
Race/Ethnicity: White, non-Hispanic	0.695	0.707	0.644
Race/Ethnicity: Black, non-Hispanic	0.069	0.07	0.062
Race/Ethnicity: Hispanic	0.117	0.107	0.16
Race/Ethnicity: Other/Multi-racial	0.068	0.07	0.061
Currently Insured	0.945	0.944	0.954
State Controls			
COVID-19 Cumulative Case Rate	0.064	0.061	0.075
COVID-19 Cumulative Death Rate	0.001	0.001	0.001
Medicaid Eligibility: Non-Parents	0.916	0.804	1.38
Medicaid Eligibility: Parents	1.061	0.984	1.38
Community Health Centers per 100k	5.939	6.447	3.831
Primary Care Physicians per 10k	6.146	5.886	7.224
Government COVID Response Index	21.515	20.931	23.931
Housing Price Index	193.611	188.118	216.377
Federal Poverty Level Ratio	289.4	286.945	299.572
Log Per Capita Income	11.011	10.979	11.144
Minimum Wage (Average)	0.46	0.439	0.545
Observations	93,875	75,626	18,249

Notes: This table presents descriptive statistics for the analytic sample drawn from the 2016–2022 waves of the National Survey of Children's Health (NSCH). All estimates are weighted using the NSCH survey sampling weights to produce nationally representative averages of children aged 12–17 years. Variables capture measures of child mental health outcomes, demographic characteristics, and state-level contextual factors. Treated states are defined as those that had an active paid sick leave (PSL) mandate in effect during the survey year.

Table 3: Estimated Effects of Paid Sick Leave Mandates on Adolescent Mental Health Care Access and Diagnoses (NSCH, 2016–2022)

	(1) Any Mental Health Care	(2) Medication for Mental Health Issues	(3) Depression Diagnosis	(4) Anxiety Diagnosis
Paid Sick Leave Mandate	0.0232**	0.0137*	0.0154**	0.0122*
	(0.00916)	(0.00695)	(0.00685)	(0.00710)
Baseline Mean	0.171	0.159	0.109	0.175
Observations	90065	89294	89866	89598
R-squared	0.029	0.021	0.033	0.037

Notes: This table reports estimates of the effect of state paid sick leave (PSL) mandates on adolescents' mental health care utilization and diagnostic outcomes. Each column presents results from a separate regression estimated using the Sun and Abraham (2021) interaction-weighted difference-in-differences framework, which accounts for staggered policy adoption and heterogeneous treatment effects. Covariates include adolescent age, sex, race/ethnicity, insurance type, household federal poverty level, parental education and marital status, household language, number of adults and children in the household, and state-level controls including the minimum wage, unemployment rate, Medicaid eligibility thresholds, health care provider supply, the Housing Price Index, log per capita income, and COVID-19 case and death rates with the Oxford Government Response Index. All models include state and year fixed effects and are weighted using NSCH survey sampling weights. Standard errors are clustered at the state level. Statistical significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.1$.

Table 4: Estimated Effects of Paid Sick Leave Mandates on Youth Depression and Suicidal Ideation (YRBSS, 2005–2021)

	(1) Depression	(2) Passive Suicidal Ideation	(3) Active Suicidal Ideation
Paid Sick Leave Mandate	-0.0219**	-0.0142***	-0.0121
	(0.00938)	(0.00462)	(0.00720)
Baseline Mean	0.303	0.165	0.135
Observations	1064699	957730	1038576
R-squared	0.052	0.022	0.016

Notes: This table reports estimates of the effect of state paid sick leave (PSL) mandates on youth self-reported mental health outcomes. Each column presents results from a separate regression estimated using the Sun and Abraham (2021) interaction-weighted difference-in-differences framework, which accounts for staggered policy adoption and heterogeneous treatment effects. Covariates include age, sex, and race/ethnicity, and state-level controls including the minimum wage, unemployment rate, Medicaid eligibility thresholds, health care provider supply, the Housing Price Index, log per capita income, and COVID-19 case and death rates with the Oxford Government Response Index. All models include state and year fixed effects and are weighted using YRBSS survey sampling weights. Standard errors are clustered at the state level. Statistical significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.1$.

Table 5: Estimated Effects of Paid Sick Leave Mandates on Adolescent Mental Health Outcomes by Subgroup (NSCH, 2016–2022)

	Age		Race/Ethnicity					Sex		Parental Education	
	(1) 12–14	(2) 15–17	(3) Asian	(4) Black	(5) Hispanic	(6) Other/Multi	(7) White	(8) Male	(9) Female	(10) No College	(11) College+
Panel A: Received Any Mental Health Care											
Paid Sick Leave Mandate	0.0209**	0.0250	0.00834	0.0312	0.00250	0.0580	0.0252**	0.0295*	0.0174**	0.0265	0.0202
	(0.00967)	(0.0207)	(0.0224)	(0.0218)	(0.0265)	(0.0348)	(0.0108)	(0.0166)	(0.00685)	(0.0181)	(0.0127)
Observations	40341	49724	4524	5969	10378	6169	63025	43486	46579	36828	53237
R-squared	0.031	0.030	0.042	0.034	0.042	0.063	0.029	0.043	0.022	0.031	0.032
Panel B: Depression Diagnosis											
Paid Sick Leave Mandate	0.00909	0.0217**	0.0237**	0.0526**	-0.0134	0.0455	0.0198***	0.00985	0.0193**	0.0109	0.0211***
	(0.00848)	(0.0107)	(0.0115)	(0.0223)	(0.0274)	(0.0385)	(0.00608)	(0.0154)	(0.00838)	(0.0138)	(0.00562)
Observations	40284	49582	4527	5950	10356	6147	62886	43378	46488	36688	53178
R-squared	0.026	0.032	0.049	0.038	0.043	0.064	0.038	0.048	0.022	0.036	0.034

Notes: Each column reports the estimated effect of state paid sick leave (PSL) mandates on the specified outcome for a given adolescent subgroup. Panel A reports effects on whether the adolescent received any mental health care in the past year, and Panel B reports effects on parent-reported depression diagnosis. Estimates are obtained using the Sun and Abraham (2021) interaction-weighted difference-in-differences estimator, which accounts for staggered policy adoption and heterogeneous treatment effects. Covariates include adolescent age, sex, race/ethnicity, insurance type, household federal poverty level, parental education and marital status, household language, number of adults and children in the household, and state-level controls including the minimum wage, unemployment rate, Medicaid eligibility thresholds, health care provider supply, the Housing Price Index, log per capita income, and COVID-19 case and death rates with the Oxford Government Response Index. All models include state and year fixed effects and are weighted using NSCH survey sampling weights. Standard errors are clustered at the state level. Estimates are suppressed for subgroup cells with fewer than 50 observations. Statistical significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$.

Table 6: Estimated Effects of Paid Sick Leave Mandates on Youth Depression and Suicidal Ideation by Subgroup (YRBSS, 2005–2021)

	Sex		Age		Race/Ethnicity				
	(1) Female	(2) Male	(3) Younger than 15	(4) 15 or Older	(5) White	(6) Black	(7) Asian	(8) Hispanic	(9) Other
Panel A: Depression									
Paid Sick Leave Mandate	-0.0243**	-0.0191**	0.00000168	-0.0249**	-0.0122*	-0.0512***	0.00495	-0.000551	0.00147
	(0.0111)	(0.00862)	(0.0110)	(0.00943)	(0.00675)	(0.0122)	(0.0648)	(0.0236)	(0.0148)
Baseline Mean	0.387	0.222	0.292	0.304	0.284	0.293	0.285	0.354	0.364
Observations	539492	525207	159454	905245	609856	136873	43857	178626	95487
R-squared	0.026	0.013	0.063	0.051	0.049	0.041	0.038	0.055	0.056
Panel B: Passive Suicidal Ideation									
Paid Sick Leave Mandate	-0.0156***	-0.0123**	-0.0196**	-0.0133**	-0.00957**	-0.0252***	-0.0115	0.00188	-0.0134
	(0.00531)	(0.00470)	(0.00891)	(0.00505)	(0.00461)	(0.00639)	(0.0273)	(0.0241)	(0.0152)
Baseline Mean	0.209	0.122	0.177	0.163	0.161	0.147	0.156	0.173	0.223
Observations	486299	471431	143193	814537	518483	134473	40404	174621	89749
R-squared	0.012	0.005	0.029	0.021	0.020	0.022	0.019	0.025	0.034
Panel C: Active Suicidal Ideation									
Paid Sick Leave Mandate	-0.0119*	-0.0119	-0.00511	-0.0129*	-0.00587	-0.0265***	-0.0175	0.00125	-0.00183
	(0.00656)	(0.00872)	(0.00758)	(0.00755)	(0.00580)	(0.00925)	(0.0117)	(0.0308)	(0.0225)
Baseline Mean	0.165	0.105	0.153	0.133	0.127	0.127	0.136	0.149	0.186
Observations	526392	512184	156526	882050	587211	136397	43171	177495	94302
R-squared	0.012	0.005	0.025	0.014	0.012	0.018	0.012	0.022	0.019

Notes: Each column reports the estimated effect of state paid sick leave (PSL) mandates on the specified outcome for a given youth subgroup. Panel A reports effects on self-reported depression, Panel B on passive suicidal ideation, and Panel C on active suicidal ideation. Estimates are obtained using the Sun and Abraham (2021) interaction-weighted difference-in-differences estimator, which accounts for staggered policy adoption and heterogeneous treatment effects. Covariates include age, sex, and race/ethnicity, and state-level controls including the minimum wage, unemployment rate, Medicaid eligibility thresholds, health care provider supply, the Housing Price Index, log per capita income, and COVID-19 case and death rates with the Oxford Government Response Index. All models include state and year fixed effects and are weighted using YRBSS survey sampling weights. Standard errors are clustered at the state level. Statistical significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.1$.

Table 7: Effects of Paid Sick Leave Mandates on Family Functioning and Parental Time Allocation (NSCH, 2016–2022)

	(1) Parent Reduced or Stopped Work	(2) Family Meals Together	(3) Parent and Adolescent Share Ideas	(4) Parent Talks with Adolescent About Concerns	(5) Family Works Together to Solve Problems
Paid Sick Leave Mandate	0.0148***	0.0612	0.00113	0.0128	0.0230
	(0.00544)	(0.0689)	(0.0116)	(0.0144)	(0.0186)
Baseline mean	0.053	4.467	0.938	0.860	0.868
Observations	89857	89914	89455	89793	89611
R-squared	0.010	0.051	0.010	0.025	0.023

Notes: This table reports estimates of the effect of state paid sick leave (PSL) mandates on family functioning and parental time allocation. Each column presents results from a separate regression estimated using the Sun and Abraham (2021) interaction-weighted difference-in-differences framework, which accounts for staggered policy adoption and heterogeneous treatment effects. Covariates include adolescent age, sex, race/ethnicity, insurance type, household federal poverty level, parental education and marital status, household language, number of adults and children in the household, and state-level controls including the minimum wage, unemployment rate, Medicaid eligibility thresholds, health care provider supply, the Housing Price Index, log per capita income, and COVID-19 case and death rates with the Oxford Government Response Index. All models include state and year fixed effects and are weighted using NSCH survey sampling weights. Standard errors are clustered at the state level. Statistical significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.1$.

Table 8: Effects of Paid Sick Leave Mandates on Care Coordination and Access (NSCH, 2016–2022)

	(1) Unmet Mental Health Need	(2) Difficulty Coordinating Care	(3) Needed Home-Based Care	(4) Trouble Getting an Appointment	(5) Facility Not Open When Needed	(6) Avoided Changing Providers	(7) Preventive Check-Up Visit
Paid Sick Leave Mandate	0.00624***	0.0210*	0.00577	0.0111***	0.00154	0.00566	0.000916
	(0.00180)	(0.0104)	(0.0585)	(0.00330)	(0.00182)	(0.00510)	(0.0185)
Baseline mean	0.013	0.174	0.302	0.019	0.007	0.061	0.737
Observations	90152	89788	89675	90106	90092	89541	89767
R-squared	0.007	0.006	0.008	0.011	0.010	0.005	0.098

Notes: This table reports estimates of the effect of state paid sick leave (PSL) mandates on care coordination and access mechanisms. Each column presents results from a separate regression estimated using the Sun and Abraham (2021) interaction-weighted difference-in-differences framework, which accounts for staggered policy adoption and heterogeneous treatment effects. Covariates include adolescent age, sex, race/ethnicity, insurance type, household federal poverty level, parental education and marital status, household language, number of adults and children in the household, and state-level controls including the minimum wage, unemployment rate, Medicaid eligibility thresholds, health care provider supply, the Housing Price Index, log per capita income, and COVID-19 case and death rates with the Oxford Government Response Index. All models include state and year fixed effects and are weighted using NSCH survey sampling weights. Standard errors are clustered at the state level. Statistical significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.1$.

Appendix Figures and Tables

Table A.1: Descriptive Statistics (YRBSS, 2005–2021)

	Full Sample	**Untreated States**	**Treated States**
Outcomes			
Self-Reported Depression	0.308	0.307	0.312
Passive Suicidal Ideation	0.168	0.167	0.169
Active Suicidal Ideation	0.137	0.137	0.139
Demographics			
Female	0.488	0.488	0.489
Age 16 or Older	0.626	0.626	0.624
Race/Ethnicity: White, non-Hispanic	0.546	0.511	0.578
Race/Ethnicity: Black, non-Hispanic	0.164	0.157	0.182
Race/Ethnicity: Hispanic	0.210	0.188	0.260
Race/Ethnicity: Asian, non-Hispanic	0.031	0.029	0.035
Race/Ethnicity: Other/Multi-racial	0.048	0.048	0.050

Notes: This table presents descriptive statistics for the Youth Risk Behavior Surveillance System (YRBSS). All estimates are weighted using the YRBSS survey sampling weights to produce nationally representative averages/proportions. Treated states are defined as those that had an active paid sick leave (PSL) mandate in effect during the survey year.

Table A.2: Stepwise Estimates of the Effects of Paid Sick Leave Mandates on Adolescent Mental Health (NSCH, 2016–2022)

	(1)	(2)	(3)	(4)
Panel A: Received Any Mental Health Care				
Paid Sick Leave Mandate	0.0200***	0.0234**	0.0235**	0.0232**
	(0.00653)	(0.00939)	(0.00988)	(0.00916)
State FE	YES	YES	YES	YES
Year FE	YES	YES	YES	YES
Demographics/HH	YES	YES	YES	YES
State Economic		YES	YES	YES
Health Supply			YES	YES
COVID Controls				YES
Observations	90065	90065	90065	90065
Panel B: Depression Diagnosis				
Paid Sick Leave Mandate	0.0137***	0.0159**	0.0158**	0.0154**
	(0.00387)	(0.00685)	(0.00721)	(0.00685)
State FE	YES	YES	YES	YES
Year FE	YES	YES	YES	YES
Demographics/HH	YES	YES	YES	YES
State Economic		YES	YES	YES
Health Supply			YES	YES
COVID Controls				YES
Observations	89866	89866	89866	89866

Notes: This table reports estimates of the effect of state paid sick leave (PSL) mandates on adolescent mental health outcomes with sequentially added controls. Panel A examines receipt of any mental health care in the past 12 months, and Panel B examines parent-reported depression diagnosis. Estimates are obtained using the Sun and Abraham (2021) interaction-weighted difference-in-differences framework with state and year fixed effects. Column (1) includes demographic and household controls only; Column (2) adds state economic conditions; Column (3) adds health care supply measures; Column (4) adds COVID-related controls. All models are weighted using NSCH survey sampling weights. Standard errors are clustered at the state level. Statistical significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.1$.

Table A.3: Stepwise Estimates of the Effects of Paid Sick Leave Mandates on Youth Reported Mental Health (YRBSS, 2005–2021)

	(1)	(2)	(3)	(4)
Panel A: Self Reported Depression				
Paid Sick Leave Mandate	-0.00291	-0.0194**	-0.0218**	-0.0219**
	(0.00689)	(0.00760)	(0.00892)	(0.00938)
Observations	1064699	1064699	1064699	1064699
Panel B: Passive Suicidal Ideation				
Paid Sick Leave Mandate	-0.000703	-0.0140***	-0.0134***	-0.0142***
	(0.00338)	(0.00491)	(0.00456)	(0.00462)
Observations	957730	957730	957730	957730
Panel C: Active Suicidal Ideation				
Paid Sick Leave Mandate	-0.00204	-0.0112*	-0.0109	-0.0121
	(0.00376)	(0.00639)	(0.00687)	(0.00720)
State FE	YES	YES	YES	YES
Year FE	YES	YES	YES	YES
Demographics/HH	YES	YES	YES	YES
State Economic		YES	YES	YES
Health Supply			YES	YES
COVID Controls				YES
Observations	1038576	1038576	1038576	1038576

Notes: This table reports estimates of the effect of state paid sick leave (PSL) mandates on youth self-reported mental health outcomes with sequentially added controls. Panel A examines self-reported depression, Panel B examines passive suicidal ideation, and Panel C examines active suicidal ideation. Estimates are obtained using the Sun and Abraham (2021) interaction-weighted difference-in-differences framework with state and year fixed effects. Column (1) includes demographic controls only; Column (2) adds state economic conditions; Column (3) adds health care supply measures; Column (4) adds COVID-related controls. All models are weighted using YRBSS survey sampling weights. Standard errors are clustered at the state level. Statistical significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.1$.

Table A.4: Estimated Effects of Paid Sick Leave Mandates on Adolescent Mental Health Care Access and Diagnoses, TWFE (NSCH, 2016–2022)

	(1) Any Mental Health Care	(2) Medication for Mental Health Issues	(3) Depression Diagnosis	(4) Anxiety Diagnosis
Paid Sick Leave Mandate	0.0216**	0.0130*	0.0165**	0.00758
	(0.00900)	(0.00687)	(0.00749)	(0.00855)
Baseline Mean	0.171	0.159	0.109	0.175
Observations	90065	89294	89866	89598
R-squared	0.029	0.021	0.033	0.037

Notes: This table reports estimates of the effect of state paid sick leave (PSL) mandates on adolescents' mental health care utilization and diagnostic outcomes using a conventional two-way fixed effects (TWFE) difference-in-differences specification. Each column presents results from a separate regression including state and year fixed effects. Covariates include adolescent age, sex, race/ethnicity, insurance type, household federal poverty level, parental education and marital status, household language, number of adults and children in the household, and state-level controls including the minimum wage, unemployment rate, Medicaid eligibility thresholds, health care provider supply, the Housing Price Index, log per capita income, and COVID-19 case and death rates with the Oxford Government Response Index. All models are weighted using NSCH survey sampling weights. Standard errors are clustered at the state level. Statistical significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.1$.

Table A.5: Estimated Effects of Paid Sick Leave Mandates on Youth Depression and Suicidal Ideation, TWFE (YRBSS, 2005–2021)

	(1) Depression	(2) Passive Suicidal Ideation	(3) Active Suicidal Ideation
Paid Sick Leave Mandate	-0.0161*	-0.0115*	-0.0139***
	(0.00831)	(0.00573)	(0.00488)
Baseline Mean	0.303	0.165	0.135
Observations	1064699	957730	1038576
R-squared	0.052	0.022	0.016

Notes: This table reports estimates from a two-way fixed effects (TWFE) difference-in-differences model examining the effect of state paid sick leave (PSL) mandates on youth self-reported mental health outcomes. Covariates include age, sex, and race/ethnicity, and state-level controls including the minimum wage, unemployment rate, Medicaid eligibility thresholds, health care provider supply, the Housing Price Index, log per capita income, and COVID-19 case and death rates with the Oxford Government Response Index. All models include state and year fixed effects and are weighted using YRBSS survey sampling weights. Standard errors are clustered at the state level. Statistical significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.1$.

Table A.6: Effects of Paid Sick Leave Mandates on Adolescent Mental Health Outcomes, Excluding COVID Year (NSCH, 2016–2022)

	(1) Any Mental Health Care	(2) Mental Health Medication	(3) Depression Diagnosis	(4) Anxiety Diagnosis
Paid Sick Leave Mandate	0.0212*	0.0119	0.0137*	0.00498
	(0.0107)	(0.00744)	(0.00811)	(0.00731)
Baseline Mean	0.172	0.160	0.109	0.175
Observations	75471	74827	75308	75102
R-squared	0.029	0.020	0.033	0.038

Notes: This table reports estimates of the effect of state paid sick leave (PSL) mandates on adolescent mental health outcomes, excluding observations from the year 2020. Each column presents results from a separate regression estimated using the Sun and Abraham (2021) interaction-weighted difference-in-differences framework. Covariates include adolescent age, sex, race/ethnicity, insurance type, household federal poverty level, parental education and marital status, household language, number of adults and children in the household, and state-level controls including the minimum wage, unemployment rate, Medicaid eligibility thresholds, health care provider supply, the Housing Price Index, log per capita income, and COVID-19 case and death rates with the Oxford Government Response Index. All models include state and year fixed effects and are weighted using NSCH survey sampling weights. Standard errors are clustered at the state level. Statistical significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.1$.

Table A.7: Estimated Effects of Paid Sick Leave Mandates on Youth Depression and Suicidal Ideation, Excluding COVID Year (YRBSS, 2005–2021)

	(1) Depression	(2) Passive Suicidal Ideation	(3) Active Suicidal Ideation
Paid Sick Leave Mandate	-0.0207**	-0.0105*	-0.00906
	(0.00798)	(0.00553)	(0.00704)
Baseline Mean	0.290	0.159	0.130
Observations	940900	851842	913804
R-squared	0.043	0.018	0.013

Notes: This table reports estimates of the effect of state paid sick leave (PSL) mandates on youth self-reported mental health outcomes, excluding the 2021 YRBSS survey wave (which covers the 2020–2021 school year). Each column presents results from a separate regression estimated using the Sun and Abraham (2021) interaction-weighted difference-in-differences framework, which accounts for staggered policy adoption and heterogeneous treatment effects. Covariates include age, sex, and race/ethnicity, and state-level controls including the minimum wage, unemployment rate, Medicaid eligibility thresholds, health care provider supply, the Housing Price Index, log per capita income, and COVID-19 case and death rates with the Oxford Government Response Index. All models include state and year fixed effects and are weighted using YRBSS survey sampling weights. Standard errors are clustered at the state level. Statistical significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.1$.

Table A.8: Effects of Paid Sick Leave Mandates on Diagnostic Scope Outcomes (NSCH, 2016–2022)

	(1) ADHD Diagnosis	(2) Autism Diagnosis	(3) Special Health Care Needs
Paid Sick Leave Mandate	0.00971	0.00288	0.0186
	(0.00658)	(0.00380)	(0.0119)
Baseline Mean	0.155	0.036	0.300
Observations	89556	89970	90419
R-squared	0.039	0.015	0.024

Notes: This table reports estimates of the effect of state paid sick leave (PSL) mandates on diagnostic scope outcomes, including parent-reported ADHD diagnosis, autism diagnosis, and children with special health care needs (CSHCN). Each column presents results from a separate regression estimated using the Sun and Abraham (2021) interaction-weighted difference-in-differences framework. Covariates include adolescent age, sex, race/ethnicity, insurance type, household federal poverty level, parental education and marital status, household language, number of adults and children in the household, and state-level controls including the minimum wage, unemployment rate, Medicaid eligibility thresholds, health care provider supply, the Housing Price Index, log per capita income, and COVID-19 case and death rates with the Oxford Government Response Index. All models include state and year fixed effects and are weighted using NSCH survey sampling weights. Standard errors are clustered at the state level. Statistical significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.1$.

Table A.9: Effects of Paid Sick Leave Mandates on Non–Time-Sensitive Child Outcomes, Placebo Tests (NSCH, 2016–2022)

	(1) Hearing Impairment	(2) Overweight / Obese	(3) Neighborhood Safety	(4) Oral Health Problems	(5) Social Skills (Make Friends)
Paid Sick Leave Mandate	-0.00430	-0.00335	0.00878	-0.00705	0.00978
	(0.00472)	(0.00855)	(0.00541)	(0.0115)	(0.00973)
Baseline Mean	0.018	0.279	0.973	0.112	0.269
Observations	90031	87918	89796	90234	89522
R-squared	0.018	0.050	0.040	0.028	0.013

Notes: This table reports estimates of the effect of state paid sick leave (PSL) mandates on outcomes plausibly unrelated to short-run parental time flexibility, used as placebo tests. Each column presents results from a separate regression estimated using the Sun and Abraham (2021) interaction-weighted difference-in-differences framework. Covariates include adolescent age, sex, race/ethnicity, insurance type, household federal poverty level, parental education and marital status, household language, number of adults and children in the household, and state-level controls including the minimum wage, unemployment rate, Medicaid eligibility thresholds, health care provider supply, the Housing Price Index, log per capita income, and COVID-19 case and death rates with the Oxford Government Response Index. All models include state and year fixed effects and are weighted using NSCH survey sampling weights. Standard errors are clustered at the state level. Statistical significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.1$.

Table A.10: Estimated Effects of Paid Sick Leave Mandates on Youth Risk Behavior, Placebo Test (YRBSS, 2005–2021)

	(1) Riding with Drunk	(2) Drunk Driving	(3) Distracted Driving
Paid Sick Leave Mandate	-0.00198	0.0000162	0.00393
	(0.00588)	(0.00186)	(0.00762)
Baseline Mean	0.220	0.038	0.241
Observations	1031660	643809	657793
R-squared	0.024	0.011	0.061

Notes: This table reports estimates of the effect of state paid sick leave (PSL) mandates on youth risk behaviors, used as placebo tests. Each column presents results from a separate regression estimated using the Sun and Abraham (2021) interaction-weighted difference-in-differences framework, which accounts for staggered policy adoption and heterogeneous treatment effects. Covariates include age, sex, and race/ethnicity, and state-level controls including the minimum wage, unemployment rate, Medicaid eligibility thresholds, health care provider supply, the Housing Price Index, log per capita income, and COVID-19 case and death rates with the Oxford Government Response Index. All models include state and year fixed effects and are weighted using YRBSS survey sampling weights. Standard errors are clustered at the state level. Statistical significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.1$.